

Synthetic Home Care Plan

For take-home assessment use only. Fictional data. Not for clinical use.

This care plan is a fictional example for software prototyping. It is designed to contain enough structure, ambiguity, and operational detail for a document-ingestion system to extract care needs, risks, goals, routines, scheduled tasks, escalation rules, and source references.

1. Person Summary

Field	Details
Care recipient	Arthur Bennett, age 78
Living situation	Lives with spouse in a two-storey home. Uses downstairs bedroom due to reduced stairs.
Primary care context	Receives morning home-care support Monday–Friday. Community nurse reviews once weekly.
Main goals	Maintain independence at home, support diabetes self-management, reduce night-time bathroom trips.
Communication preferences	Responds well to reassuring explanations and short conversational prompts. Becomes frustrated with long lists.
Review date	Fictional review date: 02 September 2026.

2. Current Needs and Risks

- Hydration: Arthur occasionally forgets to drink fluids during long periods of television watching. Intake is usually lower during warmer weather.
- Mobility: Arthur walks independently indoors with a stick but becomes unsteady when standing quickly. He should avoid carrying heavy items while walking.
- Falls: Two near-falls were reported during night-time bathroom trips over the past two months. Risk increases when lighting is poor or footwear is not worn.
- Medication adherence: Arthur usually remembers morning medication but sometimes delays evening medication if he falls asleep in the chair. The assistant must not provide dosage advice or alter medication routines.
- Diabetes monitoring: Arthur checks blood glucose most mornings but occasionally skips checks when feeling tired. The assistant may remind him about agreed routines but must not interpret readings clinically.
- Nutrition: Arthur prefers smaller meals and may skip lunch if not prompted. Offering meal choices improves engagement.
- Sleep routine: Arthur sometimes wakes multiple times overnight and naps after lunch. Late caffeine intake may worsen sleep quality.
- Mood and social connection: Arthur enjoys discussing football and gardening. Mood tends to decline during extended rainy periods when he stays indoors for several days.

3. Care Goals

- G1 Hydration: Encourage regular fluid intake throughout the day and increase awareness of hydration during warm weather.
- G2 Safe mobility: Support safe movement indoors, reduce night-time fall risk, and encourage use of stick and proper footwear.

- G3 Medication routine: Support completion of prescribed morning and evening medication checks without giving clinical advice.
- G4 Diabetes routine: Encourage agreed blood-glucose monitoring habits and routine meal timing without interpreting medical results.
- G5 Wellbeing and engagement: Encourage social interaction, enjoyable conversation, and structured daily routine to reduce isolation.

4. Routine Care Activities

Activity	Frequency / Timing	Responsible Party	Assistant Prompt Style	Source Goal
Hydration reminder	Daily mid-morning, lunch, and early evening	Care assistant	Suggests drinking casually as part of a shared activity.	G1
Morning medication check	Daily after breakfast	Care assistant	Ask whether morning medication has been taken	G3
Evening medication check	Daily between 19:00 and 21:00	Assistant or spouse	Confirm evening medication routine was completed	G3
Blood glucose routine reminder	Daily before breakfast	Conversational assistant	Gently remind Arthur about agreed glucose-check routine.	G4
Safe mobility prompt	Daily during evening movement	Conversational assistant	Remind Arthur to use hallway lighting and walking stick.	G2
Light activity encouragement	Tuesday, Thursday, Saturday	Care assistant	Encourage supervised indoor walking or stretching	G2
Nutrition prompt	Daily around lunchtime	Conversational assistant	Offer meal options and encourage light meal	G1, G4
Social engagement prompt	Weekend afternoons	Family member or conversational assistant	Encourage discussion about football or gardening	G5
Evening wind-down reminder	Daily at 22:00	Conversational assistant	Suggest dimming lights and reducing TV volume	G5

5. Escalation Guidance

- Urgent same-day human review: fall with injury, sudden confusion, severe dizziness, chest pain, significant shortness of breath, or inability to stand safely.
- Care coordinator review within 24 hours: repeated missed medication checks, skipped blood-glucose routines for two consecutive days, poor food intake across an entire day, or repeated refusal of mobility support.
- Falls-prevention review: near-falls during night-time walking, repeated walking without stick use, or movement detected in darkness without hallway lighting.
- Wellbeing review: reports of isolation for several consecutive days, unusual withdrawal from preferred conversations, or repeated disengagement from daily routines.
- Do not escalate solely from one missed reminder unless additional concerning patterns or risk indicators are present. Avoid unnecessary alarm fatigue.

6. Notes for Conversational Support

Arthur responds best to calm, respectful reminders that feel collaborative rather than instructional. The assistant should avoid sounding repetitive or overly clinical. The assistant should explain suggestions using care-plan reasoning where appropriate. Example: “Your care plan notes that using the hallway light at night helps reduce fall risk, so I’m checking whether the light is on before you walk.” The assistant must never diagnose conditions, provide medication changes, interpret glucose readings clinically, or replace a healthcare professional.

7. Example Questions the System Should Support

- What should Arthur be reminded about this evening?
- What care tasks are scheduled for today?

- Why are you suggesting a hydration reminder?
- What risks should the care team monitor this week?
- Explain the diabetes support routine simply.
- Mark the evening medication check as completed.
- Should repeated missed glucose checks be escalated?
- What wellbeing activities are recommended this weekend?

8. Known Ambiguities for the Prototype

Some care-plan elements are intentionally ambiguous for extraction and reasoning purposes. For example: • “reduced appetite” • “if Arthur feels steady” • “social withdrawal” • “poor hydration” should not trigger automatic clinical conclusions. The assistant should preserve uncertainty, explain reasoning clearly, and request human confirmation where necessary.